



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ياسر عبدالمجيد حسن حسين

العنوان : ش الوحدة

الحى / المدينة : امبابة

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 06-10-2026

رقم البطاقة : 26303140101291 رقم التليفون : 01147969955

العمر (تاريخ الميلاد) : 14-03-1963 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ياسر عبدالمجيد حسن حسن National ID: 26303140101291	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

ياسر
عبدالمجيد حسن حسين
٦١ ش الوحدة
اميلية - الجيزة

١٩٦٣/٠٣/١٤ ٢٦٣٠٣١٤٠١٠١٢٩١

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كياوى

٢٠٢٥/٠٧ ٢٦٣٠٣١٤٠١٠١٢٩١

حاصل على ليسانس دارالعلوم

ذكر مسلم متزوج

البطاقة سارية حتى ٢٠٣٢/٠٧/٠١

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Patient Name : الأستاذ / ياسر عبد المجيد حسن	Request No. : 13005826
Patient No. : 26303140101291	Request Date : 11/04/2026
Path No. : SR-448/26	Verification Date : 05/05/2026
Sex : Male	Financial Category : نفقة الدولة - جوستاف روسي
Age : 63 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left male breast cancer.

Nature of Specimen: Slide Revision

Gross Examination:

- One paraffin block labeled by (96387) and one H&E stained slide, Ten paraffin blocks labeled by (1033951) and ten H&E stained slides were received.

Microscopic Examination:

Examination of the core biopsy:

- * Histological type: Invasive duct carcinoma
- * Grade 2 (differentiation3+ nuclear pleomorphism2+ mitosis1)
- * DCIS: Absent
- * Angioinvasion: Absent
- * Perineural invasion: Absent
- * Additional pathologic findings: Necrosis.

Examination of the MRM specimen:

- * Histological type: Invasive duct carcinoma
- * Grade 2 (differentiation3+ nuclear pleomorphism2+ mitosis1)
- * DCIS: Absent
- * Angioinvasion: Present
- * Skin: Tumor infiltrating the skin with ulceration (nipple and areola skin).
- * Resection margins: Free margins.
- * Lymph node involvement: Positive for metastasis in (2/15) with extracapsular extension.

DIAGNOSIS

Left breast cancer TCNB, Slide revision:

Invasive duct carcinoma, Grade 2.

Verified by :

Dr Maha Guimie



Patient Name : الأستاذ / ياسر عبد المجيد حسن
Patient No. : 26303140101291
Path No. : SR-448/26
Sex : Male
Age : 63 Y
Reffering Doc :
Request No. : 13005826
Request Date : 11/04/2026
Verification Date : 05/05/2026
Financial Category : جوسف روسي - نفقة الدولة
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Left breast MRM Specimen (slide revision)
Left breast cancer, Slide revision:
- Invasive duct carcinoma, Grade 2.
- Infiltrating the skin with ulceration (nipple and areola skin).
- Axillary lymph nodes: Positive for metastasis in (2/15) with extracapsular extension.
- Margins.
- Stage T4b N1
For further IHC on both (core biopsy and MRM) (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahim Ghoneim

Verified by :
Dr Maha Guimie

Printing Date : 13/05/2026 12:25:49

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building Nos 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الاستاذ / ياسر عبد المجيد حسن :	Request No. : 13005826
Patient No. : 26303140101291	Request Date : 11/04/2026
Path No. : SR-448/26	Verification Date : 05/05/2026
Sex : Male	Financial Category : جوسفان روسي - نفقة الدولة
Age : 63 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer
Nature of Specimen: Slide revision

Technique:

Sections were prepared from the paraffin blocks of core biopsy and MRM specimen, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view RP DAB as chromogen and hematoxylin as counter stain. The sections treated against: SISH, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

Core biopsy:

IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 98% of tumor cells (8/8).
PR: Strong nuclear reaction in 80% of tumor cells (8/8).
Her2: Moderate complete membrane reaction in > 10% of tumor cells (score 2+).
SISH: Not amplified. (Average Her2 copy No.: 2.8, Her2/CEP17 ratio: 1.43)
Ki67: 25% nuclear reaction of tumor cells.

Left breast, MRM specimen (slide review)

IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 98% of tumor cells (8/8).
PR: Strong nuclear reaction in 60% of tumor cells (7/8).
Her2: Moderate complete membrane reaction in > 10% of tumor cells (score 2+).
SISH: Not amplified.
Average Her2 copy No.: 2.8, Her2/CEP17 ratio: 1.43)
Ki67: 25% nuclear reaction of tumor cells.

Verified by :

Dr Maha Guimie



Patient Name الاستاذ / ياسر عبد المجيد حسن
Patient No. 26303140101291
Path No. SR-448/26
Sex Male
Age 63 Y
Referreing Doc.

Request No. 13005826
Request Date 11/04/2026
Verification Date 05/05/2026
Financial Category جوستاف روسي - نفقة الدولة
Department Out Patient
Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Conclusion:
Core Biopsy:
Left breast cancer, Slide revision:
ER: Positive (score 8/8).
PR: Positive (score 8/8).
Her2: Equivocal (score 2+)
SISH: Not amplified.
Ki67: Positive in 25% of tumor cells.

MRM Specimen, Slide revision:
ER: Positive (score 8/8).
PR: Positive (score 7/8).
Her2: Equivocal (score 2+)
SISH: Not amplified.
Ki67: Positive in 25% of tumor cells.

Scoring system: "ER, PR"
SCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 7%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane

Verified by :
Dr Maha Guimie

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Page 2 of 3

Printed By : .marwa mohamed yousse



Name : Yaser Abdel-Majid

Date : 05-07-2025 09:54

Branch : Maadi

Medical No. : 142996

Ref. Physician Prof : Abdelrahman Albana

Ins. Company : Walkin

II) Distant:

- No distant FDG-avid focal lesions or pathological lymph nodes could be seen all over the surveyed body.

III) Rest of the body:

- No apparent brain focal lesions or pathologic tracer distribution.
- No FDG-avid pulmonary nodules, masses or infiltrates.
- No pleural or pericardial effusion, thickening or masses.
- Enlarged fatty liver with no FDG-avid focal lesions. No intra or extra-hepatic biliary radicle dilatation.
- Normal appearance of the spleen, pancreas, kidneys, suprarenals and pelvic structures.
- No pelvi-abdominal ascites or fluid collections.
- No significant FDG-avid osseous foci, sclerotic or lytic lesions.

IV) Side findings:

- Calcular gallbladder.
- Spondylodegenerative changes are noted over the spine.





VILE
N & LABS

مركز النيل للأشعة والتحاليل

Name : Yaser Abdel-Majid

Branch : Maadi

Ref.Physician Prof : Abdelrahman Albana

Date : 05-07-2025 09:54

Medical No. : 142996

Ins.Company : Walkin

V) Normal physiological tracer bio-distribution is seen over the brain, bone marrow, glandular structures, heart, bowel and urinary tract.

Conclusion:

- Positive whole body (FDG) PET/CT study for metabolically active left breast retro-areolar soft tissue mass lesion; matching the proven active tumor biology, with no metabolically active lesions to account for locoregional or distant metastatic deposits all over the surveyed body.
- Follow-up whole body (FDG) PET/CT will be advisable as clinically warranted.

As Regards,

*Dr. Moumen Katamesh, M.Sc
Nuclear Medicine*

*Dr. Yasser Mohamed
Professor of Nuclear Medicine*

Dr. : Yaser Abdul Majid
Tech : Nasreldy

Date : 02-02-2026 10:28

Medical No. : 307178

Whole body (FDG) PET/CT Study:

Clinical history:

- 63-year-old male patient with history of managed left breast cancer.
- PET/CT is requested for follow up.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5 .

PET/CT findings:

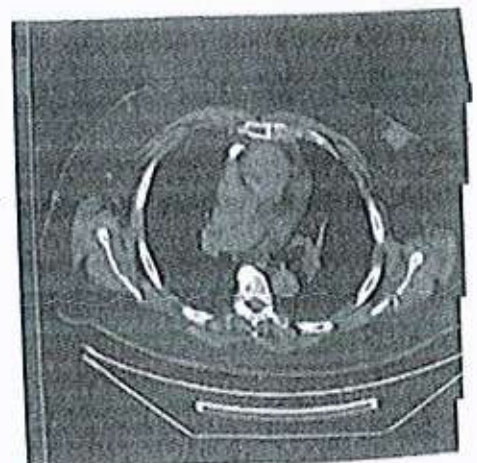
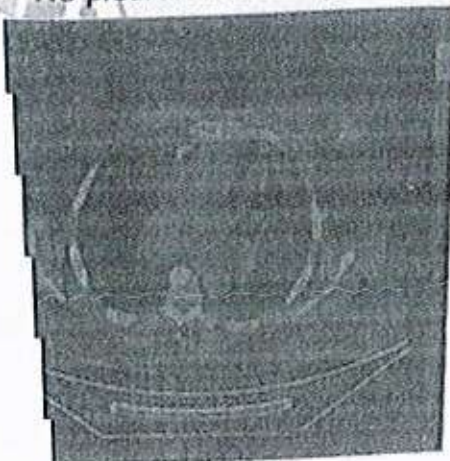
Compared to previous PET/CT study dated: 05/07/2025.

Head and neck:

- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.
- No pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.

Chest:

- Regressive morphologic course of the prior reported FDG-avid left breast retro-areolar soft tissue mass lesion, currently measuring ~ 1.8 cm; Vs. 2.4 cm and eliciting the same degree of tracer uptake with SUV max ~ 3.9 as before
- No definite lung nodules or parenchymal infiltrates.
- No enlarged or tracer-avid axillary, mediastinal or hilar LN.s.
- No pleural thickening, masses or effusion.



Name : Yaser Abdel-Majid

Branch : Maadi

Ref. Physician Prof : Abdelrahman Albana

Date : 05-07-2025 09:54

Medical No. : 142996

Ins. Company : Walkin

Whole Body (18F-FDG) PET/CT Study

History and Clinical Data:

Patient data:

- o Weight: 118 Kg, Blood Glucose level: 144 mg/dl.

Clinical background:

- o 62-year-old patient with a recently proven left breast cancer via mass biopsy on 18/06/2025.

Radiopharmaceutical:

- o 185 MBq of 18F-FDG was injected I.V.

Purpose of the study:

- o Initial assessment.

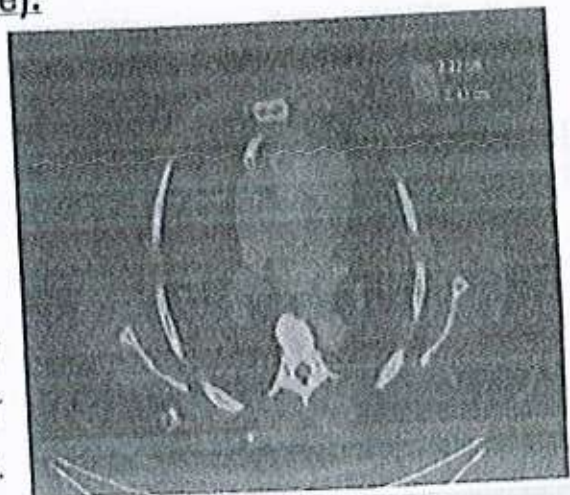
Technique:

- o Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- o **Contrast Enhanced CT** images done for attenuation correction and anatomic localization were obtained followed by PET images from the vertex of the skull to the thighs.
- o PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- o The Standardized Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed.
- o Reference Hepatic SUV max ~ 3.9 and SUV mean ~ 2.3.

Findings:

I) Locoregional (Both breasts and axillae):

- o FDG-avid left breast retro-areolar soft tissue mass lesion, measuring ~ 2.2 x 2.4 cm and eliciting SUV max ~ 3.9.
- o The right breast is unremarkable.
- o No significant FDG-avid or pathologically enlarged axillary or other mammary-related lymph nodes.





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Fellow, Armed Forces Institute of Pathology , USA
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Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ياسر عبد المجيد حسن حسين

Analysis ID : 1033951

Age : 62 Y : 1 M : 2 D

Gender : Male

Conf. Code : 6567

Referred By Prof. Dr. : محمود عبد النبي

Clinical Diagnosis : Left breast retroareolar mass.

Nature of Specimen : Modified radical mastectomy.

Receiving Date : 2026-02-13

Patient ID : 714391

PATHOLOGY REPORT

Diagnosis :

Left breast mass, Modified radical mastectomy, INFILTRATING DUCT CARCINOMA
GRADE II, NO INTRADUCTAL COMPONENT, FREE SURGICAL MARGINS, AXILLARY
LYMPH NODE METASTASIS , L.N. 2 / 16 WITH EVIDENCE OF EXTRANODAL
EXTENSION.

- PATHOLOGIC PARTIAL RESPONSE TO THERAPY (R2).
- TUMOR STAGE: ypT4b, ypN1a.
- RESIDUAL CANCER BURDEN = 4.111.
- RESIDUAL CANCER BURDEN CLASS: III.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

DR ELIA ANIS LABS

This report is electronically signed , any abrasion or modification would render it void .

2/2

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تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

PET-3-17
[[DOCTYPE:PET_SCAN]]





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Dr.

Shady Elia Anis

M. Sc. MD Pathology, Asst. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : **ياسر عبد المجيد حسن حسين**

Age : 62 Y : 3 M : 7 D

Referred By Prof. Dr. : **عبد الرحمن البها**

Clinical Diagnosis : Left breast retroareolar mass

Nature of Specimen : Block (963875)

Receiving Date : 2025-06-22

Analyte ID :  665574

Gender : Male

Patient ID : 714391

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .

The sections were treated against:

- Estrogen receptors - Progesterone receptors - Her 2 - Ki67.

Results :

95 % of tumor cells show STRONG POSITIVE Staining for Estrogen receptors.

85% of tumor cells show STRONG POSITIVE Staining for progesterone receptors.

Sections treated for HER2 revealed MINIMAL INCOMPLETE FAINT CELL MEMBRANE STAINING IN MORE THAN 10% OF TUMOR CELLS.

20 % of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE;

POSITIVE FOR ESTROGEN RECEPTORS (SCORE 8 / 8).

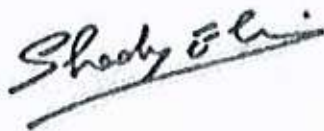
POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 8 / 8).

HER 2 LOW, SCORE 1+.

THE PROLIFERATION INDEX OF Ki67 IS 20%.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak





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Dr.

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M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : **ياسر عبد المجيد حسن حسين**

Age : 62 Y : 3 M : 3 D

Referred By Prof. Dr. : **عبد الرحمن الهللا**

Clinical Diagnosis : Left breast retroareolar mass.

Nature of Specimen : U/S guided tru-cut biopsy.

Receiving Date : 2025-06-18

Analysis ID :  963875

Gender : Male

Patient ID : 714391

PATHOLOGY REPORT

Gross :

Multiple greyish white and yellowish tissue cores, the longest measured 1.5 cm long, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed cores of tumour tissue formed of malignant ductal epithelial cells, showing moderate nuclear pleomorphism and hyperchromasia. There is moderate attempt at tubular formation.

There are moderate number of mitotic figures.

There is moderate desmoplasia with microcalcification.

There is peritumoural lymphocytic infiltrate around 5%.

There is focal necrosis.

No intraductal component.

Diagnosis :

Left breast retroareolar mass, U/S guided tru-cut biopsy, INFLTRATING DUCT CARCINOMA, GRADE II.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

This report is electronically signed, any obliteration or modification would render it void.

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عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤١٧١٢ - ٢٥٧٧٢٢٥٢ - ٢٥٧٩٠٩٣٥ - ٢٥٧٩٠٥٢١٧

